



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نبيه محمود محمد الغمراوي

المحافظة : القاهرة الحي / المدينة : المعادي العنوان : الجيزة

رقم البطاقة : 25106101202795 رقم التليفون : 01224669710 تاريخ تسجيل الإستمارة : 04-10-2026

العمر (تاريخ الميلاد) : 10-06-1951 المهنة : طالب

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : ورم خبيث بالبروستاتا

نتائج الأبحاث : (مرفق)

التشخيص النهائي : ورم خبيث بالبروستاتا

خطة العلاج : أورام كيماوي العلاج الموجه بعقار (Abirateron acetate) كخط علاج قسم العلاج : الأول في حالات MHSPC [14510] الأورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نبيه محمود محمد الغمراوي National ID: 25106101202795	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of prostate cancer for First-line targeted Abiraterone acetate therapy for MHSPC
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

مملكة البحرين
وزارة الداخلية
مطابق تقرير الشرطة

تاريخ

محمود محمد الغفراني

٢٣٣١ على السودان - المهنة

المجوزة - الجيرة

٢٠١٠٦٩٠١٣

٢٠١٠٦٩٠١٣



٢٠٢٤/٠٨

٢٥١٠٢١٠١٢٠٢٧٩٥

بالتسليم

مقروء

مسلم

نكر

٢٠٢٤/٠٨/٠١

General MDT form

Patient Name: أحمد محمد العبدى	Age: 75 Y
Sex: Male	Special Habit: no
ID: 25106101202795	Medical ID: 9554
Family History:	Comorbidity: -ve
Surgical History:	Telephone: 01001799883
Menopausal Status:	Marital Status:
Referring Physician: MDT	Reason Of MDT: new case
MDT Name: Thorecic	
Cancers / GU Cancers/ GIT	

Medical Report

Diagnosis:	
Pathology: prostatic adeno. GS 3+3.	
IHC:	
T:	N:
M: 1	
History:	
Radiology Investigation:	
Recommendations: start 1st line Zoladex+xyziga+Zometa every 3/6 month , prostate gland: Adenocarcinoma, NOS Male pt 75 yrs old , -ve MH , -ve FH ,	
MDT DECISION: AS PT HSPC FPR ZOLDEX /CASODEX FOR FLARE PHENOMENA ZOMETA / 6MONTHS ADD Zeteca through NGO IF AVAIL	

Signature	Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry
	<i>أحمد محمد العبدى</i>	<i>أحمد محمد العبدى</i>	<i>أحمد محمد العبدى</i>			

DR. AHMED YOUSSEF MONTASSER

Head of Pathology Department, TBRI
M.D. of Pathology, Cairo University
Fellow, Beaujon University Hospital, France
Member, European Society of Pathology
Member, USCAP
Member, ISUP



د. أحمد يوسف منتصر

رئيس قسم الباثولوجي - معهد تبودور ناهارس
دكتورة الباثولوجي - جامعة القاهرة
زميل مستشفى بوجون الجامعي - فرنسا
عضو الجمعية الأوروبية للباثولوجي
عضو الأكاديمية الأمريكية للكندية للباثولوجي

Name: نبيه محمود محمد

Age: 75 Y

Sex: Male

Referred by: Prof. Dr. Alaa Nouh

Signed out: 12-02-26

Lab ID: 26AM0402

Prof. Dr. Ahmed Tawfik

PATHOLOGY REPORT

Clinical Data: PSA >100ng/ml. DRE: Hard. TRUS: Diffuse mixed echotexture.

Nature of Specimen: TRUS-guided biopsies.

Gross Features: Six containers were received, each with white a core, 10mm in average length.

Microscopic Features:

Sections examined from Left Apex and Apical Horn biopsies (1) revealed a core of prostatic tissue totally infiltrated by tumor tissue composed solely of well-formed discrete acini lined by a single layer of cubical cells having large nuclei with high nucleocytoplasmic ratio and focally prominent nucleoli.

Sections examined from Right Apex and Apical Horn biopsies (2) revealed a core of prostatic tissue showing hyperplastic and atrophic acini with preserved double lining surrounded by hyperplastic fibromuscular stroma.

Sections examined from Left Mid Prostate Lateral, Anterior biopsies (3) revealed a core of prostatic tissue totally infiltrated by tumor tissue composed solely of well-formed discrete acini lined by a single layer of cubical cells having large nuclei with high nucleocytoplasmic ratio and focally prominent nucleoli.

Sections examined from Right Mid Prostate Lateral, Anterior biopsies (4) revealed a core of prostatic tissue focally showing a gland with complex cribriform structure, lined by cubical cells with mildly pleomorphic nuclei with no central necrosis. The rest of the presented core showed hyperplastic and atrophic acini with preserved double lining.

Sections examined from Left Base Postero-Medial, Lateral biopsies (5) revealed a core of prostatic tissue totally infiltrated by tumor tissue composed solely of well-formed discrete acini lined by a single layer of cubical cells having large nuclei with high nucleocytoplasmic ratio and focally prominent nucleoli. Perineural invasion was seen.

Sections examined from Right Base Postero-medial, Lateral biopsies (6) revealed a core of prostatic tissue minimally infiltrated by tumor tissue composed solely of well-formed discrete acini lined by a single layer of cubical cells having large nuclei with high nucleocytoplasmic ratio and focally prominent nucleoli.

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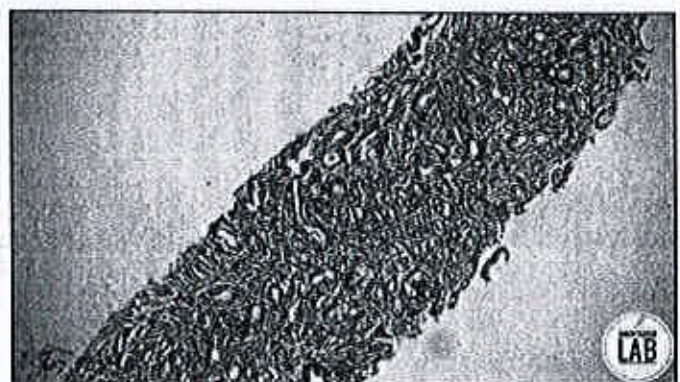
contact@amontasserlab.com



Opinion:

Prostate, TRUS-Guided Biopsies,

- 1- Left Apex and Apical Horn, Prostatic Acinar Adenocarcinoma, Gleason Score 3+3= 6, Grade Group 1, Involving 100% of the Presented Core.
- 2- Right Apex and Apical Horn, Benign Prostatic Hyperplasia, Atrophy.
- 3- Left Mid Prostate Lateral, Anterior, Prostatic Acinar Adenocarcinoma, Gleason Score 3+3= 6, Grade Group 1, Involving 100% of the Presented Core.
- 4- Right Mid Prostate Lateral, Anterior, Focal Atypical Intraductal Proliferation.
- 5- Left Base Postero-medial, Lateral, Prostatic Acinar Adenocarcinoma, Gleason Score 3+3= 6, Grade Group 1, Involving 100% of the Presented Core.
- 6- Right Base Postero-medial, Lateral, Prostatic Acinar Adenocarcinoma, Gleason Score 3+3= 6, Grade Group 1, Involving Less Than 10% of the Presented Core.



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دكتوراه الباثولوجي - جامعة القاهرة
زميل مستشفى بوجون الجامعي - فرنسا
عضو الجمعية الأوروبية للباثولوجي
عضو الأكاديمية الأمريكية الكندية للباثولوجي

Synoptic Report Summary (Case 26AM0402)

According to CAP Cancer Reporting Protocol v 1.1.0.0 (Sep 2023)

Procedure	TRUS-guided biopsies
Positive specimen(s)	Sites labeled 1, 3, 5, 6
Histologic type	Acinar adenocarcinoma
Highest Gleason score	Grade group 1 (Gleason score 3+3= 6)
Perineural Invasion	Present
Periprosthetic fat invasion	Not Identified
Intraductal carcinoma	Not Identified
Tumor quantification:	
Total number of cores	6
Number of positive cores	4
Greatest % of core involvement	100% (Cores labeled 1, 3, 5)

Signature

Ahmed Montasser, MD
ANATOMIC PATHOLOGIST

3/3

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Name : Nabih Mahmoud Mohammed Al-Ghamrawi	Date : 21-02-2026 09:10
Branch : Nasrelty	Medical No. : 320132
Ref.Physician Prof : هشام الغزالي	Ins.Company : نقابة تجارين القاهرة

Whole body PET/CT EXAMINATION:

Clinical History:

- 75 years old patient diagnosed with prostate ADC.
- PET/CT requested for initial staging assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5 .

Report:

No previous PET-CT available for comparison.

Hepatic Reference SUVmax~1.8.

Head and Neck:

- Normal diffuse brain activity with no hypermetabolic brain lesions (N.B: it may kindly be noted that some brain lesions not to be apparent on PET/CT and MRI is better for local assessment, if clinically warranted).
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.
- Normal appearance of the neck muscles & vessels.
- No hypermetabolic cervical lymphadenopathy detected.
- No FDG avid neck soft tissue mass lesions.

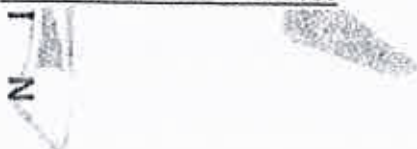
Chest:

- No hypermetabolic mediastinal or axillary lymphadenopathy.
- No FDG avid lung nodules or masses detected apart from mild bilateral pleural thickening and non FDG avid Rt LLL cystic like lesion~3.7 x 5.3 cm for clinical correlation and FU.



- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:



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- * Enlarged prostate with rather heterogenous diffuse low grade FDG avidity of SUVmax~1.8 and subtle focal lesion at Lt peripheral zone~1.6 cm for further correlation with multiparametric MRI, serum PSA level and histopathological data.
- * Associated with multiple low grade FDG avid abdomino-pelvic para-aortic, aortocaval, Lt common/ internal iliac LNs, the largest measures~2.3 x 3.4 cm of SUVmax~2.4 at the earlier.



- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected. Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicle dilatation.
- * Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No FDG avid ascites or abdominal collections noted.

Musculoskeletal:

- * Diffuse heterogenous low grade FDG avid mixed skeletal osseous changes/ lesions seen at most of the examined axial and proximal appendicular skeleton eliciting SUVmax~3.7.



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Branch : Nasrcity	Medical No. : 320132
Ref.Physician Prof : هشام الغزالي	Ins.Company : نقابة تجاريبين القاهرة



Conclusion:

PET-CT study shows:

Enlarged prostate with rather heterogenous diffuse low grade FDG avidity and subtle focal lesion at Lt peripheral zone for further correlation with multiparametric MRI, serum PSA level and histopathological data.

Associated with multiple low grade FDG avid abdomino-pelvic LNs, for close FU. Worrisome diffuse heterogenous low grade FDG avid mixed skeletal osseous changes/ lesions for close FU.

Thank you for referral
Dr. Nada Fadl, MD
Nuclear Medicine
Dr. Walid Omar, MD.
Prof. of Nuclear Medicine

N I L E D I A G N O S T I C



Collection Date: 02/10/2026
Review Date: 02/10/2026
Referring Doctor: أ.د محمد اسماعيل

Patient Name: الأستاذ/ نبيه محمود محمد الغمراوي
Patient No.: 208-5239
Accession No.: 208261015646
Age: 75Y

IMMUNOLOGY UNIT

Test Name	Result	Unit	Reference Interval
Free PSA, Serum	>18.3	ng/mL	
Total PSA, Serum	>1460	ng/mL	Up to 4.0
Comments :	PSA value must be interpreted in conjunction with clinical evaluation and other diagnostic procedures. Follow up is recommended.		



Reviewed By:

T. S. Hassan

Pt. Branch: الشيخ زايد - ازار بلازا

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16191

Lab Test

[[DOCTYPE:LAB_TEST]]



Collection Date: 02/10/2026

Review Date: 02/10/2026

Referring Doctor: أ.د محمد اسماعيل

Patient Name: الأستاذ/ نبيه محمود محمد الغمراوي

Patient No.: 208-5239

Accession No.: 208261015646

Age: 75Y

IMMUNOLOGY UNIT

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Reviewed By:

T. S. Hassan

Pt. Branch: الشيخ زايد - ازار بلازا

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16191

Lab Test

[[DOCTYPE:LAB_TEST]]

Name : Nabeeh Mahmoud Mohammed Al-Ghamrawi	Date : 21-09-2026 07:17
Branch : Dokki	Medical No. : 466337
Ref.Physician Prof : Hesham El Ghazaly	Ins.Company : نقية تجاريل القاهرة

Follow-up Whole Body (18F-FDG) PET/CT Study

History and Clinical Data:

Patient data:

◦ Weight: 55 Kg, Blood Glucose level: 93 mg/dL.

Clinical background:

◦ 75-year-old patient with managed metastatic prostate cancer.

Radiopharmaceutical:

◦ 111 MBq of 18F-FDG was injected I.V.

Purpose of the study:

◦ Follow-up in comparison to the prior (FDG) PET/CT study dated 21/02/2026.

Technique:

- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- Non-Contrast Enhanced CT images done for attenuation correction and anatomic localization were obtained followed by PET images from the vertex of the skull to the thighs.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardized Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.
- Reference Hepatic SUV max ~ 2.5 compared to SUVmax ~ 2.3.

Findings:

I) Prostate:

- Still noted enlarged prostate indenting the inferior vesical wall with rather stationary heterogeneous low grade parenchymal metabolic activity, eliciting SUVmax up to 1.9 compared to SUVmax up to 1.8 within the posterior para-median peripheral/transitional zones.
- No FDG-avid focal lesions could be seen within either seminal vesicle.

II) Nodal (regional and distant):

- Metabolic and morphologic regression of the priorly seen multiple enlarged low grade metabolically active bilateral external and to a lesser extent internal iliac lymph nodes, the largest one is seen at the left external iliac group measuring ~ 1 x 1.5 cm compared to ~ 2 x 3.2 cm with SUVmax ~ 1.1 compared to SUVmax ~ 2.1.
- Metabolic and morphologic regression of the priorly seen multiple enlarged low grade metabolically active bilateral common iliac, para-aortic, aorto-caval lymph nodes, the largest one is seen at the proximal left common iliac group measuring ~

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Date : 21-09-2026 07:17

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- 0.9 x 1.8 cm compared to ~ 2.5 x 3.8 cm with SUVmax ~ 1.1 compared to SUVmax ~ 2.4.
- No significant FDG-avid or pathologically enlarged regional or distant lymph nodes.

III) Distant (extra-nodal):

- Stationary few bilateral pulmonary nodules with insignificant metabolic activity, the largest left side one is seen within the superior lingular segment of upper lobe measuring ~ 0.8 x 0.9 cm with SUVmax ~ 0.5, while the largest right side one is seen within the medial aspect of posterior segment of upper lobe measuring ~ 0.6 x 0.6 cm with SUVmax ~ 0.5 (priorly seen, yet not reported).
- Metabolic and morphologic progression (regarding size, number and sclerotic burden) of the priorly seen rather diffuse/widespread low grade metabolically active osseous lesions implicating the most deal of axial and appendicular skeleton, currently eliciting SUVmax up to 5.5 at the intertrochanteric region of right femur compared to SUVmax up to 4.1 at the posterior aspect of right iliac bone.

IV) Rest of the body:

- Age-matched brain involutional changes with no apparent focal lesions or pathologic tracer distribution.
- Heterogenous thyroid parenchyma with **stationary** prominent right lobe posterior hypodense nodule (measuring ~ 1 x 1.8 cm with weak metabolic activity of SUVmax ~ 1.1) with small calcified focus.
- **Stationary** non-metabolically active well-defined hypodense/cystic lesion within the anterolateral portion of the right lower lobe with diaphragmatic indentation, measuring ~ 3.5 x 5.5 cm.
- **Stationary** mild bilateral lower posterior costal pleural thickening with no significant metabolic activity, for further follow-up.
- Mildly enlarged fatty liver with heterogeneous parenchymal texture, yet no FDG-avid focal lesions. No intra or extra-hepatic biliary radicle dilatation.
- **Stationary** relatively enlarged left suprarenal gland with no apparent focal lesions or pathological tracer uptake; likely representing simple glandular hyperplasia.
- Normal appearance of the spleen, pancreas, kidneys, right suprarenal and other pelvic structures.
- No pelvi-abdominal ascites or fluid collections.

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V) Side findings:

- Spondylodegenerative changes are noted over the spine.
- Atherosclerotic calcifications are noted within the aorta and bilateral iliac arteries.
- VI) Normal physiological tracer bio-distribution is seen over the brain, bone marrow, glandular structures, heart, bowel and urinary tract.

Conclusion:

- Follow-up whole body (FDG) PET/CT study shows:
- Stationary enlarged prostate with heterogeneous low grade parenchymal metabolic activity.
- Metabolic and morphologic regression of the multiple low grade metabolically active regional and distant (pelvi-abdominal) lymph nodes.
- Stationary few bilateral pulmonary nodules with insignificant metabolic activity.
- Metabolic and morphologic progression of the widespread sclerotic osseous deposits.
- No metabolically active lesions elsewhere.
- Kindly note that 18F-PSMA whole body PET/CT will be of a good added value in further assessment in such case, for your final decision.

As Regards,

Dr. Moumen Katamesh, MSc
Nuclear Medicine Specialist
Dr. Yasser Mohamed
Professor of Nuclear Medicine

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Ref.Physician Prof : Hesham El Ghazaly

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